Surgery for vulval cancer

You may have surgery to treat vulval cancer. The surgery you have will depend on the size and position of the cancer.

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About vulval cancer surgery

The aim of surgery for vulval cancer is to remove all the cancer. The surgeon will remove the area of skin where the cancer is, and a border (margin) of healthy tissue all around it. In the same operation, the surgeon may also:

- remove some nearby lymph nodes to check them for cancer cells (sentinel lymph node biopsy)
- remove lymph nodes affected by cancer from one or both sides of the groin
- remove lymph nodes that are likely to contain cancer from one or both sides of the groin.

What to expect before surgery

Your surgeon will talk with you about your operation and what it will involve. They will explain if you are likely to need further operations to reconstruct the vulva. A nurse specialist will also give you information and support before and after your operation.

It is important to ask any questions and talk about any worries you have about the surgery.

What to expect after surgery

Your team will also explain what to expect after surgery. Some operations may change how the vulva looks and feels. They can sometimes affect your sex life or your ability to pass urine (pee) or stools (poo). If you have a lot of lymph nodes removed, this can affect fluid drainage from the legs and vulva. It can cause swelling called lymphoedema in one or both legs.

It is important to understand the possible changes and the support your team can give you to help you cope.

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Types of vulval surgery

There are different operations to treat cancer of the vulva, depending on the size and position of the cancer. If the cancer is very small, it may be possible to remove only a small area of the vulva. If the cancer is larger, you may need a bigger operation.

Wide local excision

This operation is sometimes called a radical local excision. The surgeon takes away the cancer and a border (margin) of healthy tissue all around the cancer. The margin is usually at least 1cm wide. This helps to lower the risk of the cancer coming back.

You may have lymph nodes removed from one or both sides of the groin as well, depending on where the cancer is.

Image: Wide local excision

Partial vulvectomy

If the tumour is larger, more of the vulval skin will be removed. This is to make sure there is a border (margin) of healthy tissue all around the cancer. This operation is sometimes called a radical partial vulvectomy.

You may have lymph nodes removed from one or both sides of the groin as well, depending on where the cancer is.

Surgery to remove the front area of the vulva is called an anterior vulvectomy. This operation sometimes involves removing part of the clitoris or the end of the tube that drains urine from the bladder (the urethra).

The following diagrams show different types of radical partial vulvectomy.

Image: Anterior vulvectomy

Image: Posterior vulvectomyImage: Hemi-vulvectomy

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Vulvectomy

This operation removes the whole vulva, including the inner and outer labia and the clitoris. It is also called a radical or total vulvectomy.

The lymph nodes are also removed from one or both sides of the groin.

Image: Total vulvectomy

Stoma

For some people surgery will involve having a stoma. A stoma is an opening made during the operation through the tummy (abdomen) wall. It lets the bowel connect to the surface of the tummy. Stool no longer passes out of the rectum and anus in the usual way. Instead it passes out of the stoma, into a disposable bag that is worn over the stoma.

You may need a stoma:

- if the cancer has affected the anus or rectum (back passage) and your surgery involves removing these areas
- to manage symptoms until you have had other treatments such as chemotherapy or radiotherapy.

It can take time to adjust to having a stoma. If you need to have one as part of your surgery, a stoma nurse will give you more information.

Reconstructing the vulva

Depending on the area that is removed, you may need to have surgery to reconstruct the vulva. This is usually at the same time as the operation to remove the cancer. But it can also be done later.

If only a small amount of skin is removed from the vulva, the surgeon may be able to stitch the remaining skin neatly together. Or they may leave it to heal naturally. This will also depend on the site of the cancer.

If a larger area of skin is removed, you may need to have a skin graft or skin flap.

Skin grafts

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The surgeon will take a piece of skin from another part of the body (usually the thigh or buttock) and place it over the area where the cancer was removed. The place where the skin is taken from is called the donor site. The place where it is moved to is called the grafted area. The amount of skin taken depends on the area to be covered. Your doctor or specialist nurse will tell you more about this.

Skin flaps

A skin flap is a slightly thicker layer of skin than a graft. It is taken from an area close to the vulva. The flap is cut away, but left partially connected so it still has a blood supply. It is moved over the wound and stitched in place. If you have a skin flap, you may need to stay in hospital for a few days.

Skin flap surgery is very specialised. You may have to travel to a different hospital to have it. If you need a skin flap, your doctor will be able to tell you more about it.

Pelvic exenteration

You will usually be offered a combination of radiotherapy and chemotherapy. if the cancer has spread to organs close to the vulva. This includes the:

- womb
- bladder
- lower bowel.

It may still be possible to have an operation to remove the cancer. This involves a major operation called a pelvic exenteration, where any affected organs are removed.

Recovery following this type of operation can be difficult both physically and emotionally. So it is not done very often. But sometimes this operation may be the only way to completely remove the cancer.

About our information

Reviewers

This information has been written, revised and edited by Macmillan Cancer Support's Cancer Information Development team. It has been reviewed by expert medical and health professionals and people living with cancer. It has been approved by Senior Medical Editor, Professor Nick Reed, Consultant Clinical Oncologist.

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Our cancer information has been awarded the PIF TICK. Created by the Patient Information Forum, this quality mark shows we meet PIF's 10 criteria for trustworthy health information.

The language we use

We want everyone affected by cancer to feel our information is written for them.

We want our information to be as clear as possible. To do this, we try to:

- use plain English
- explain medical words
- use short sentences
- use illustrations to explain text
- structure the information clearly
- make sure important points are clear.

We use gender-inclusive language and talk to our readers as 'you' so that everyone feels included. Where clinically necessary we use the terms 'men' and 'women' or 'male' and 'female'. For example, we do so when talking about parts of the body or mentioning statistics or research about who is affected.

You can read more about how we produce our information here.

Date reviewed

Reviewed: 30 April 2021 | Next review: 30 April 2024

This content is currently being reviewed. New information will be coming soon.

Our cancer information meets the PIF TICK quality mark.

This means it is easy to use, up-to-date and based on the latest evidence. Learn more about how we produce our information.

How we can help

Macmillan Support Line

The Macmillan Support Line is a free and confidential service for people living with and affected by cancer. If you need to talk, we'll listen.

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Call us free on 0808 808 0000

8am to 8pm, 7 days a week. You can talk to our cancer specialists about anything.

Email the Macmillan Support Line

Email the Macmillan Support Team if you have any questions.

Chat online

Available 8am to 7.45pm, 7 days a week. For confidential, real-time support if you don’t want to call.

Online Community

Chat online anonymously to others who understand what you are going through. Our community is available 24/7 and has dedicated forums where you can get advice and ask our experts.

Find cancer support services in your area

What's going on near you? Find out about support groups, where to get information and how to get involved with Macmillan where you live.

Get help

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